AyusCare Health Center

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Patient Name	: **Miss. KARISHMA**	Reg/Lab No.	: 26060135 / PR141387
Age/Sex	: 9 Yrs / Female	Second Date Of Colllection 1	: 17-06-2026 at 12:19 PM
Contact No.	: 1234567890	Date of Report	: 19-06-2026 at 12:37 PM
Ref. By.	: **Dr. Bharath**		

TEST PARAMETER	RESULT	UNIT	NORMAL RANGE
VITAMIN D 25 HYDROXY			
VITAMIN D 25 HYDROXY *ECLIA*	**27.31** (L)	ng/mL	Deficiency: <10 ng/mL Insufficiency: 10 - 30 ng/mL Sufficient: 30 - 100 ng/mL Toxicity: >100 ng/mL

Note:

- *The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D.*
- *25 (OH)D is influenced by sunlight, latitude, skin pigmentation, sunscreen use and hepatic function.*
- *Optimal calcium absorption requires vitamin D 25 (OH) levels exceeding 30 ng/mL.*
- *It shows seasonal variation, with values being 40-50% lower in winter than in summer.*
- *Levels vary with age and are increased in pregnancy.*
- *This is the recommended test for evaluation of vitamin D intoxication.*

Comment: *Vitamin D (Cholecalciferol) promotes absorption of calcium and phosphorus and mineralization of bones and teeth. Deficiency in children causes Rickets and in adults leads to Osteomalacia. It can also lead to Hypocalcemia and Tetany. Vitamin D status is best determined by measurement of 25 hydroxy vitamin D, as it is the major circulating form and has longer half life (2-3 weeks) than 1,25 Dihydroxy vitamin D (5-8 hrs).*

Decreased levels:

- *Inadequate exposure to sunlight.*
- *Dietary deficiency.*
- *Vitamin D malabsorption.*
- *Severe Hepatocellular disease.*
- *Drugs like Anticonvulsants.*
- *Nephrotic syndrome.*

Increased levels:

- *Vitamin D intoxication.*

L- Low, H- High

Dispatched by: Ayus Demo

**** End of Report ****

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